

CHAPTER 8

In-Sight and Direct Access

We can enter the internal system through many doors. Some are esoteric, like shamanic rituals and medicines that affect the chemical system of the brain such as MDMA (known as Ecstasy as a street drug), psilocibin (a hallucinogenic substance in certain mushrooms), Ketamine (an anesthetic that is now being used to treat depression), or its electrical system (some form of neurofeedback), other doors involve the arts (e.g., drawing or painting and movement) or some form of therapy (e.g., sand tray, psychodrama, movement, talk). When the client is adult, the IFS model relies primarily on directing the client's attention with techniques we call in-sight and direct access, which we describe in this chapter and which have been used extensively by many therapists at this point. Although we rely on in-sight and direct access, if the therapist is inclined to offer a range of options some clients respond well to methods that externalize parts. Sand tray, drawing, and movement, for example, are frequently used with children. These methods are easily integrated with IFS. Whatever method you choose, we advise you to proceed carefully, keeping in mind the pitfalls we describe in this chapter, which are relevant regardless of methodology.

Early in therapy the client's consciousness is often "up for grabs," with parts blending and unblending as they struggle for influence and control. Parts who have not been acknowledged, spoken to, or validated (sometimes ever) may not seem to be (and perhaps do not experience themselves as) particularly differentiated from each other. As a result, the client's inner experience can seem noisy, baffling, and pointless. Most clients, however, find they can follow a sensation, feeling, or thought to this inner realm, where they can get into communication with individuated parts remarkably fast. After this shift, and after the client chooses a target part, the question is how to proceed. Who is going to talk to whom?

WHO TALKS TO WHOM?

If the client’s Self is available, it does the communicating, which we refer to as *in-sight*. However, when the client’s Self is not available the therapist’s Self talks to the client’s parts directly, without the client’s Self as the intermediary: hence, *direct access* (see [Box 8.1](#)). The therapist can also invite parts to talk to one another using direct access, or they can talk to the client’s Self. Gestalt therapy’s open-chair technique is one way of conducting direct access, but most often in IFS we don’t have to guide the client to move; we simply ask to talk to a part directly, as we describe in this chapter. Whether we use in-sight or direct access, we aim to avoid part-to-part interactions between therapist and client; we want to lead with our Selves, not our parts. Finally, since our first mission is to learn about this individual’s inner community, but we do not want to barge into the client’s psychic home, we are careful to ask for permission at each step when using either in-sight or direct access.

BOX 8.1. Communication Options: What We Do and Do Not Want		
In-sight (Client’s Self is available)	Direct access (Client’s Self is not available)	Avoid (Client’s Self is not available)
Client’s Self to client’s parts	Therapist’s Self to client’s parts	Client’s parts to client’s parts (unless invited by the client’s or the therapist’s Self)
Therapist’s Self to Client’s Self		Therapist’s parts to client’s parts
(Therapist’s Self is available)	(Therapist’s Self is available)	(Therapist’s Self is not available)

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IN-SIGHT

I (RS) stumbled upon the method of communicating with parts that I named *in-sight* when my clients reported seeing their parts and watching them interact. Subsequently I learned that Jung had made a similar discovery and developed a similar approach (Hannah, 1981), a technique he called “active imagination.”

For those who are visual, a lot of inner communication comes in the form of imagery. Nevertheless, imagery is not the best term for in-sight because in IFS the client is not making an effort or being told to imagine anything. While some clients can see their parts with great clarity right away, others sense or see their parts only vaguely, and a minority (like me, RS) are not

internally visual and can't see anything inside. Those who are not internally visual generally communicate with their parts kinesthetically or aurally, which can be disorienting for individuals who are used to navigating the world by eye. In any case, when we focus internally we seem to be able to enter and see (or feel or sense) a world that already exists. The word *in-sight* is therefore intended to signify the “keen understanding” that comes from inner inquiry, whether that experience is visual in nature or not.

Clients who are blocked from seeing inside at the outset of therapy tend to be dominated by one of several kinds of protectors, including highly rational, analytic, striving parts who worry about performance, parts who do not trust the therapist, and parts who don't trust the IFS approach. When clients cannot see their parts, we go with the flow. If we are confident in the model and curious about the client's experience, the client's managers will relax eventually and inner vision may develop. But visuals are not, in any case, a prerequisite for using *in-sight*. Clients who never see their parts can still sense and interact with them. *In-sight* only requires the willingness to be internally attuned.

WITH IN-SIGHT THE CLIENT BECOMES THE THERAPIST

With the initial help of the therapist many clients can identify a large number of parts and help them differentiate from the Self quickly, after which the client's Self is able to communicate with the client's parts. In effect, the client's Self takes over the role of therapist, and the therapist serves as an adjunctive guide and witness. Once clients know how to use *in-sight* they can practice between sessions to boost their confidence and decrease their dependence on the therapist. Clients who can get enough access to their Self usually find *in-sight* efficient. When a client can't get access to the Self we use direct access as we describe next.

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DIRECT ACCESS

In direct access the therapist communicates with client parts without the client's Self acting as the intermediary. The therapist's Self can talk to parts of the client directly, or can facilitate the client's parts talking to one another, or can talk to the client's Self (e.g., *Can I talk directly with the part who puts you to sleep during sessions? Okay. I want to speak to the part who falls asleep during therapy. Are you there?*). If the client has a history of extreme trauma, direct access may be the best option and can function as the primary door to the inner world for quite a while.

Direct access has several advantages. When clients have been severely hurt or scared their protectors may have little trust in anyone and may therefore need to vet the therapist by talking to her directly. A survivor of chronic incest, for example, may reject the concept of untapped inner resources (the Self) out of hand. When protectors will not allow the client's Self to embody, efforts to try to place the client's Self in the lead won't work. In this instance, direct access is not just more efficient than in-sight it is also safer because it offers wary protectors the opportunity to develop a relationship with the Self of the therapist, who can help them resolve their polarizations much as the Self of the client would. This help piques their curiosity about the client's Self and prepares them to switch to in-sight when they feel more trusting.

Thus, when protectors block the client's Self, we act as the Self for the client's system. With direct access we spend much of each session talking with one part or another. We may also help two parts talk to each other. Although talking to parts directly looks similar to the approach described in the literature on dissociative identity disorder, it differs in one important respect. As the client's parts begin to trust the therapist, the therapist suggests that they open space inside for the client's Self to emerge and, when they finally agree, the therapist shifts leadership to the client's Self, starting with needs that arise between sessions. In contrast, DID therapists do not know about the Self, so they, in essence, do direct access with clients for the whole treatment.

Direct contact can be challenging because highly reactive parts will reveal any fault lines in the therapist's system. Therefore, when we sit with dramatically polarized clients we only attempt direct access if we can lead with the Self regardless of a wide range of provocations. If we fail to maintain Self-leadership under assault from an extreme part the best option is to own up and apologize. Any attempt to cover for our protectors will diminish the client's trust. That said, in our experience direct access is a great opportunity to get to know some powerful parts, and the interaction can be as rewarding for the therapist as for the part.

Finally, direct access allows parts to embody and express themselves fully, and we benefit by seeing parts in action. When a part embodies, the client's tone, posture, and movement all change, providing a graphic view of her inner personalities and impressing even skeptical therapists as a vivid, *in vivo* illustration of psychic multiplicity. Direct access also helps parts learn about their feelings and thoughts by putting them into words. Students of IFS may want to familiarize themselves with the concept of parts by taking some time to do nothing but direct access with a few of their least polarized clients.

DIRECT ACCESS IN COMBINATION WITH IN-SIGHT

For many clients, direct access is a good complement to in-sight (see [Box 8.2](#)). Before moving to in-sight we often use direct access to talk to a client's managerial parts about what we plan to do and how safe it will be, which gives managers the opportunity to check us out. Once we have their permission to proceed, we shift to in-sight. But we can still move back to direct access whenever doing so might work better. If, for example, a part (or a whole group of parts) is highly activated and the client's Self isn't getting any purchase we can ask to speak to the target part directly. Additionally, if at any point in therapy we intuit that progress would be expedited by a wary part having personal contact with the therapist's Self, we can ask to speak to that part directly. Overall, therapy goes more smoothly when concerned protectors have the opportunity to speak, clarify their goals, and feel understood.

BOX 8.2. When and Why to Use Direct Access

USE DIRECT ACCESS TO ...

- # Talk to managerial parts about the plan for therapy.
- # Give protectors an opportunity to have a relationship with the therapist.
- # Speak with highly activated parts, or parts whom the system fears.
- # Talk to a part who won't talk to the client.
- # Facilitate the unblending process.

BENEFITS OF DIRECT ACCESS

- # Parts can embody to express themselves fully.
- # Parts can learn about their feelings and thoughts by putting them into words.
- # Protectors are able to clarify their motives and goals.
- # Protectors connect with the therapist's Self, and also feel more connected to the client's Self, which is listening.
- # As other parts watch the direct access interview, they come to see the interviewee (the part) differently, fearing or hating it less and being more willing to include it.
- # All of this can help many parts be willing to unblend.

DISADVANTAGES OF DIRECT ACCESS

- # Inefficient, slower.
- # Risk of talking to a part without protector permission.
- # Can stimulate transference, which can be more taxing for the therapist.
- # Doesn't promote parts' attachment to the client's Self.

THE DISADVANTAGES OF DIRECT ACCESS

Direct access has three disadvantages. The first is inefficiency. It is, generally speaking, slower than in-sight, particularly if we are helping a whole group of parts. Second, there is a risk that we will violate the system's rules by talking to a part without permission. Direct access with highly polarized, conflict-ridden systems is a delicate operation. We may feel tempted to talk to an exile (or an exiled protector) before the system can tolerate its emergence. When we talk to one part directly, it's harder to be sure that we have full permission from the rest of the system (conversely, once we talk with a "hot" part, the rest of the system is likely to feel less afraid of it). Third, with direct access, parts are building an attachment to the therapist rather than to the client's Self, which is fine as long as this is temporary and the client's Self comes in as soon as possible.

DIRECT ACCESS AND EXILE OVERWHELM

Exiles have the ability to choose whether they will or will not overwhelm the client emotionally. This is one of the most important discoveries in IFS. If a client is engulfed in scary thoughts or memories, or suddenly turns into a sobbing, terrified, desperate child in the middle of a session, it is likely that an exile is breaking through to consciousness and overwhelming him emotionally. If the therapist believes episodes like this are a sign of pathology, her fear may lead her to shoo the part away with grounding skills or some technique that can have a managerial tone, sending a message to the part that it isn't welcome and needs to leave.

We view the emergence of terror or desperation as the sign of a part in need of help, not of deeper pathology. This perspective is central to IFS. In the face of panic, flashbacks, dissociation, desperate weeping, and other intense behavior, we ask permission to talk directly to the part. Then we speak to it (using direct access) in a soothing, kind, respectful way, asking about its situation and encouraging it to let the client's Self help by separating even just a little bit. Most of the time exiles cooperate and the client is quickly grounded. The importance of the therapist's Self-leadership in such situations cannot be overemphasized. Because the threat of emotional overwhelm by an exile activates protectors and obscures the client's Self, negotiations that aim toward unblending in order to tone down the client's emotional state are almost always conducted via direct access, with the therapist's Self talking to the client's parts.

As mentioned previously, researchers Singer and Klimecki (2014) explained that the act of *feeling with* can either involve a deep sense of resonance and connection (what we're calling mature

empathy), or it can tip into emotional overwhelm and cause social withdrawal (which they call *emotion contagion* and *empathic distress*). In contrast, *feeling for* (compassion) is associated with positive, other-oriented thoughts and feelings as well as prosocial behavior. Consequently, *feeling for* (compassion) develops internal stability, whereas *feeling with* (the continuum from empathic distress to mature empathy) has the potential to be internally destabilizing, depending (in IFS terms) on how internally differentiated parts are from the Self. Singer (personal communication, November 2017) also found that compassionate *feeling for* manifests in a different neural network (reward circuitry) than the empathic *feeling with* (pain circuitry) response. In our experience, the *feeling with* of a part can nicely inform the *feeling for* of the Self without becoming emotionally overwhelming, and we hypothesize that this is mostly likely to occur when the part has a good relationship with the Self.

Conversely, when we as therapists fear and avoid our own exiles, leaving them burdened, they activate empathically in response to our clients' exiles, which in turn motivates our protectors to try to control our clients (and us). Our goal, therefore, is to be mindfully present for our own exiles and heal them. When our parts feel cared for, we can empathize and lead with the Self. When we lead with the Self, we feel connected to and deeply caring for others, and more than that we have the clarity to see and hear their feelings without the distortions of our own projected pain; we feel calm and curious about how to help; and we have the confidence, creativity, and courage to act effectively on their behalf (see [Box 8.3](#)).

BOX 8.3. Empathy and Compassion in IFS Language

- # **Burdened part-to-burdened part:** (*being* the other) emotion contagion, empathic distress, emotional overwhelm
- # **Unburdened part-to-unburdened part:** (emotional resonance and *feeling with* the other) empathy
- # **Self-to-part:** (*feeling for* the other with care and concern) compassion
- # **Self-to-Self:** compassionate, heartfelt, empathic attunement and connectedness

STABLE CONNECTEDNESS

Ultimately in IFS we aim for stable connectedness on several tracks. The first is connectedness between the Self and parts (Self-to-part), which is characterized by compassion and facilitates inner balance and harmony; the second is between the Self of one person and the Self of another person (Self-to-Self), which is characterized by compassion and kindness; and the third is between parts

who have let their burdens go (unburdened part-to-unburdened part). Unburdened part-to-part relationships are characterized by emotional resonance, and stand in sharp contrast to the contagion, overwhelm, distress, and protector reactivity that are endemic in the relationships of burdened parts. Our job is to offer exiled, distressed parts the attention they crave in return for not overwhelming the Self—an offer they rarely refuse. Asking exiles not to overwhelm the Self is one way of solving the great problem of dysregulated emotion in psychotherapy.

BEGINNING DIRECT ACCESS

Direct access is not difficult. As with in-sight, this simple method begins with the client focusing internally. When the client identifies a sensation, feeling, or thought we guide her to focus on it as a part and ask whether the part would be willing to speak with the therapist directly. If the part agrees, the subsequent exchange would go something like the following.

THERAPIST: Just let the part be you, and you can also watch.

MARCELA: Seems strange, but I'll try.

THERAPIST: Okay. I'll ask questions. If it doesn't want to talk, that's okay. We'll do something else.
Ready?

MARCELA: Yes.

THERAPIST: I want to talk to the distrusting part of Marcela. Are you there?

MARCELA: *(After a pause, nods.)*

THERAPIST: *(continuing)* What do you say to Marcela?

[To differentiate the part from Marcela, the therapist asks a question about how the part relates to Marcela.]

MARCELA'S PART: I tell her not to trust anyone.

As we see, a relationship-focused question leads the part to speak of itself as separate from Marcela. [Box 8.4](#) lists various ways that these initial questions can be phrased.

BOX 8.4. Some Initial Direct Access Questions for the Target Part

- # What do you say (or do) to Marcela?
- # Why do you say (or do) that to Marcela?
- # What do you make Marcela think (or do)?
- # What do you do for Marcela?

Who do you see when you look at Marcela?
 # How do you think Marcela feels toward you?
 # What are you afraid would happen if you stopped doing this to Marcela?
 # How old do you think Marcela is?
 # How old are you?

After the part answers, the therapist goes on to the same kinds of questions we use with in-sight. These questions range broadly from the part's real intentions and its desired new role to the ways in which it would like the Self to help. Finally, after getting whatever information seems pertinent, the therapist asks whether the part wants to add anything and thanks it for coming forward. Then we ask the client to comment on the experience. Although most clients express interest, some find watching the therapist talk to one of their parts (often a part who will not talk to them) weird. If the client believes this is a sign of mental illness, we revisit the topic of psychic multiplicity, emphasizing that it is normal to have parts, and for parts to be willing to talk to the therapist before they will talk with the client. Let's continue with Marcela's distrustful part.

THERAPIST: What's the most important thing about Marcela not trusting anyone?

MARCELA'S PART: Being safe.

THERAPIST: That makes sense. I know Marcela has been unsafe a lot. What do you say to her about other people?

MARCELA'S PART: I tell her that people are bad and will hurt her. And I tell her to believe me because she's no different. She's bad, too.

THERAPIST: Does she believe you?

MARCELA'S PART: Most of the time.

THERAPIST: What are you afraid would happen if you stopped saying that to Marcela?

MARCELA'S PART: She's young and stupid. It wouldn't be a good idea.

THERAPIST: How old are you?

MARCELA'S PART: I'm older than she is.

The therapist might want to continue to question this part and get to know more about it, but if he felt an opening he could also ask the part if it would like to meet Marcela's Self. If the part says no to the latter, the therapist can revert to asking questions that flesh out the part's role, motivations, goals, and fears, along with why it doesn't want to get to know the Self. When the conversation with the part feels complete, the therapist asks to speak with Marcela.

THERAPIST: Thank you for telling me all of this. Again, given Marcela's history, your distrust of people makes complete sense to me, and we are going to rely on you to decide when you want to meet the Marcela who is not a part. I also understand why it is hard to trust me and I want us to keep talking about that. I see my job as working to earn your trust, and I get that might take some time. Is it okay if I talk to Marcela again now?

MARCELA'S PART: Okay.

THERAPIST: Are you there, Marcela? Did you hear all that?

MARCELA: Yes.

THERAPIST: How do you feel toward this part now?

MARCELA: I actually appreciate how it's trying to keep me safe. I want to help it.

As we hear at the end of this segment of direct access, by listening to the therapist talk to this forceful protector Marcela now understands it better and appreciates it more, which means that a little bit of unblending has occurred, even though the client's Self was not directly involved. This level of separation, along with her appreciation, will expedite their relationship.

TWO-CHAIR TALKS

Many clients do quite well sitting in one chair the whole time and relating to their parts without externalizing them. For some clients, however, externalizing parts can be very helpful, in which case we can facilitate a dialogue between a part and the client's Self with the "two-chair" technique of Gestalt therapy by designating one chair for a part and, across from this, another chair for the Self. Embodying each in turn, the client moves between the two chairs and they converse until their relationship makes progress.

To create a dialogue between two parts, or among several, we can use the same procedure. After accessing parts and assigning them to chairs, the client can move between chairs and speak from each part. A client can also make slight postural moves while staying in one chair, or can move sideways on a couch to designate the switch between parts. Meanwhile, our role is to engage with these parts in just the way a Self-led family therapist would engage with external family members. We befriend them and include the client's Self as soon as possible. However they are conducted, chair dialogues are not the only option for externalized communication. IFS therapists have been known to use many other externalizing techniques with adults as well as children, including sand tray, finger puppets, decks of cards with IFS-based illustrations, drawing and mapping parts, and various forms of psychodrama.

CONCLUSION

In this chapter we have described and illustrated the two IFS strategies for communicating with parts: in-sight and direct access. In-sight involves the client's Self communicating with the client's parts internally, whereas direct access involves the therapist's Self talking to the client's parts directly while the client listens. Both are valuable and effective. That said, when a client's parts have very little trust in the Self, or when the Self is not available, we mostly use direct access, which helps us to help clients manage strong emotions without using grounding practices that can send vulnerable parts the wrong message. At the same time, direct access helps parts trust that unblending and letting the client's Self return will be a safe experience with many benefits.